BPS Approved Certificate in Clinical Supervision

The British Psychological Society
Approved

Essential Supervision Skills

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SDS Seminars Ltd

Essential Supervision Skills

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Course Aims

- Examine different definitions of supervision and its raison d'etre
- Critically examine models of supervision and explore their practical implications
- Enable delegates to practice skills associated with supervision
- Critically examine practical issues around structuring and running supervision sessions
- Meet the standards required to confer eligibility to the Society's Register of Applied Psychology Practice Supervisors

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A Brief History Of Supervision

- Traditional apprentice model then adopted by medical profession mid 19th century
- Parallel development with social work overseeing volunteers
- Russell Sage Foundation (1911) offered 1st course in social work supervision
- Adopted by Freud easily 20th century : case-based group instruction
- By 1920s training analysis (with supervisee analysing 1-2 patients under supervision of an experienced colleague + own analysis+ analysts self use of word association tests)
- Spread to associated fields: mental health, child work & other psychotherapies
- 1943 Dennis O'Neill, 1973 Maria Colwell,1984 Tyra Henry, 2003 Victoria Climbié
- 1991 Beverley Allitt (NHS)
- 1997 Clinical Governance (NHS)
- 1995 onwards Registration/Accreditation and Regulation

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Exercise

State in a single sentence what you believe the purpose of supervision is

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Aims and Functions

- Ensures safe and effective practice (Falender & Shafranske, 2004)
- Fosters treatment fidelity (Inman et al., 2014)
- Maximize outcomes for clients (Callahan et al., 2009)
- Offers support to supervisees (Knudsen et al., 2008)
- Foremost 'signature' method and most critical part of practice skills training (Watkins & Milne, 2014)
- Is typically required for the (re)accreditation of professional training and/or organisation
- Accepted defence against litigation (Knapp & VandeCreek, 1997)
- Higher job satisfaction, reduced staff turnover (CQC 2013)

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Definitions

The formal provision, by approved supervisors, of a relationship-based education and training that is work-focused and which manages, supports, develops and evaluates the work of colleague/s

Milne (2001)

"An intervention provided by a more senior member of a profession to a more junior colleague…typically (but not always)…of that same profession.[Its] evaluative and hierarchical and extends over time….enhancing the professional functioning of the more junior…monitoring the quality of…services offered to the client

Bernard & Goodyear (2014)

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Supervision Functions

Derived from Proctor (1987)

Developmental	Normative	Supportive
Development of supervisee's practice skills, knowledge, insight and confidence	Helping the supervisee to ensure their practice is of an appropriate standard and that key benchmarks are met	"Holding/containing" supervisee, giving emotional support and encouragement

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Counselling & Counselling Psychology | Counselling | Psychoanalyst Child Psychology | Educational Psychology | Coaching HR backgrounds | Coaching HR background

Operationalised?	Bernard & Goodyear (1992)	Fall & Sutton (2002)	Falender & Shafranske (2004)	Hart (1982)	Hawkins & Shohet (1989)	Loganbill et al (1982)
1.Senior person	✓	✓	X	✓	X	✓
2.Relationship	X	X	✓	X	✓	✓
3.Educational	X	✓	✓	✓	✓	X
4.Longitudinal	✓	X	X	X	X	X
5.Evaluative	✓	✓	✓	✓	X	X
6.Quality control	✓	✓	✓	X	X	X
7.Gatekeeping role	✓	✓	X	X	X	X
8.Objectivity of role	X	✓	X	X	X	X
9.Supportive	X	X	X	X	X	X
10.Experienced	X	✓	✓	X	X	X
11.Dev competence	X	✓	✓	✓	X	X
12.Science informd	X	X	✓	X	X	X
13.Dev confidence	X	X	✓	X	X	X
Supported By Evidence?						
Consensus claimed	X	X	X	X	X	X
Literature support	X	X	X	X	X	X
1+ Empirical study	X	X	X	X	X	X

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"The most problematic issue surrounding the contemporary idea of clinical supervision in nursing both here and overseas, concerns what it actually is"
Yegditch (1998)

"The lack of consensus on a definition of Clinical Supervision is the most serious obstacle to developing the field further."
Buus et al. (2011)

"To this day there remain significant differences in what is meant by the term "supervision", resulting in a surprisingly diverse range of practices"
Milne (2018)

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Dissatisfaction with Supervision

Supervisors' lack of clarity regarding nature & purpose (Lister & Crisp 2005)

35% of supervisees surveyed had experienced "Harmful" supervision Ellis (2014)

1 in 4 Social workers rated supervision as poor
69.2% felt emotional issues were not covered sufficiently
62% felt personal development and training not properly covered (BASW 2012)

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Professional Body Statements

BACP

'A formal arrangement for therapists to discuss their work regularly with someone who is experienced in both therapy and supervision. In some settings supervision may also incorporate elements of training and assessment. Whatever the chosen format, the supervisory relationship is at the heart of effective supervision (Millar 2007; Stafford 2008). The supervisor's goal is "to do whatever seems most likely to send the other person away more aware, more informed, skilled and encouraged than she was when she came in." (Houston 1990)

'A working alliance between the supervisor and counsellor in which the counsellor can offer an account or recording of her work; reflect on it; receive feedback and where appropriate, guidance. The object of this alliance is to enable the counsellor to gain in ethical competence, confidence, compassion and creativity in order to give her best possible service to the client' (Inskipp & Proctor) (2012)

'Supervision is a formal but collaborative process that involves regular 'consultative support', and in which two or more people form a 'supervisory alliance' with shared objectives about how to work together constructively to provide a safe, ethical and competent service to clients' (BACP, 2018)

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Professional Body Statements

Counselling Psychology (DCoP statements)

'Supervision support is a contractually negotiated relationship between practitioners for the purpose of supporting, evaluating and developing professional practice' (2005)

'An 'activity', a 'process', a 'relationship' and a 'practice'….it is a professional and ethical activity for reflection on the work that allows for 'playful reflection' for the purposes of future action and is distinct from therapy…a process of 'ongoing collaborative, experiential and transformational learning' that draws on practice and research based knowledge. It is a 'flexible' relationship of 'mutual trust, respect and integrity' that takes into account the learning needs of the supervisee. Finally, it is a practice that is bounded by an explicit contract and draws on 'shared and explicit models of supervision' (2007)

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Professional Body Statements

Psychotherapists (UKCP statements)

Supervision is a process conducted within a formal working relationship in which a Psychotherapeutic Practitioner or Trainee presents their client work to a designated supervisor as a way of enhancing their practice through careful reflection and reflexive practice on the process. Experienced (as defined by Colleges) psychotherapeutic practitioners may choose to use peer supervision to meet this purpose.

Supervision can take place in facilitated groups, peer groups, on a one- to-one basis, by telephone, online, in writing, verbally or by use of digital media). Appropriate modes of supervision will need to be determined by the circumstances; and using different methods (e.g. live observation, digital recordings, written and verbal reports) (2018)

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Professional Body Models

Clinical Psychology (DCP statements)

'…to maintain the quality of performance, and to extend an individual's range of skills' (1995)

'…it will be sensible to …eschew making unduly prescriptive recommendations that cannot be reasonably supported by the available evidence. Gently does it shall be our credo…' (2003)

"The primary purpose of supervision is to ensure the safety and quality of care and treatment for service users. Supervision also supports professional development, developing and embedding new skills and ensuring adherence to good practice both in clinical and professional areas….. At a time of ongoing change within services, the supervisory function has a particular role to allow the individual practitioner to reflect on the personal impact of their work and manage concerns in order to assist them in maintaining their level and standard of functioning (2014)

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Professional Body Models

Occupational Therapy (BAOT / RCOT statements)

'A professional relationship which ensures good standards and encourages professional development' (1990)

'Supervision is not counselling….Supervision is not the equivalent of performance review and responsibility for work carried out' (1997)

''The definition of supervision has matured over time, taking on additional aspects or qualities, …but it can be difficult to find one common …use of these. At its simplest, supervision is a professional relationship and activity which ensures good standards of practice and encourages development.' (2015)

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Professional Body Models

' HCPC requires….registrants to understand the importance of …..supervision and ….recognise the value of ….supervision and other methods of reflecting on and reviewing practice HCPC (2013)

There are 3 interrelated aspects to supervision: Line management…. Professional supervision (case supervision) ….A key aspect of this function is reviewing and reflecting on practice issues….Continuing professional development of workers Skills For Care (2007)

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Professional Body Models

Nursing (CQC, RCN, NMC and NHSBT)

We do not….advocate any particular model of clinical supervision and we do not provide detailed guidance about its nature and scope.' NMC (2006)

Clinical supervision advocated (but not mandated) by Care Quality Commission (2013), the Royal College of Nursing (2017) and supports the Nursing and Midwifery Council (NMC) revalidation requirements

"Any nurse who feels confident and competent enough to be a supervisor can take on this role" (2018)

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Professional Body Models

Social Work England

' Supervision is the support and guidance you get from a supervisor, manager, mentor, or
peer. It involves:

- discussing ongoing cases and issues arising from them
- reflecting on and learning from practice
- supporting and maintaining wellbeing
- reviewing practice & decision making…..[is] in line with legal and statutory guidance

Supervision can look different depending on workplaces and settings. For example, this could be peer supervision, or you could be supervised by another professional' (2021)

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Social Work Survey

(BASW 2012)

- Frequency: 58.7% 1x mth / 1x every 2mths 16% and nearly a quarter 1x every 3 mths or less
- 1 in 4 rated supervision as poor
- 69.2% felt emotional issues were not covered sufficiently
- 62% didn't think personal development and training not properly covered

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Common trends in supervision models

- Supervision models typically derive from psychotherapy / counselling field
- Supervision models focus on explaining different things e.g. function, style, content etc
- Majority are theoretically / ideologically driven rather than outcome / evidence based driven
- Three types of model:
  - Pragmatic supervision specific model eg Proctor
  - Developmental model eg. Stoltenberg & Delworth
  - Therapy modality model (or hybrids) eg. Milne (CBT), Hawkins & Shohet (psychodynamic)

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Does clinical supervision "work"?

"There is very little research that investigates the supervisory experiences of qualified mental health professionals in general (and)… as yet… limited empirical support." (DCP 2003)

"…there is insufficient research… as to… outcomes. It can only be hoped that a body of research based evidence about supervision will be available in the not too distant future." (Lewis 2012)

"…there is very little evidence on the positive impact of supervision to a counsellor's wellbeing" BACP (2021) after Schofield and Grant (2013)

"…while supervision is meant to ensure safe and effective practice, it cannot always guarantee good client outcomes" Reiser & Milne (2014)

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Butterworth et al (1996/97)

23 site study across England and Scotland

Harris Nurse Stress Index
Cooper Coping Skills Questionnaire
Maslach Burnout Inventory
General Health Questionnaire
The Minnesota Job Satisfaction Scale

Retrospective
Poor control groups

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Brunero & Stein-Parbury (2008)

- Review of 22 outcome studies of effects of clinical supervision in nursing
- 16 of the studies had subject numbers 6-34 only
- Outcome measures self report only and primarily related to restorative function

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Wheeler & Richards (2007)

Systematic review of supervision for counsellors

N= 426 / N= 27/ N=1 (client outcome) !!!

- Outcome exclusively based on supee report ONLY
- No independent outcome measures

Watkins (2011)

Review of all research 1981-2006 on impact of supervision "the collective data appeared to shed little new light on the matter"

Cutcliffe, Sloan & Bashaw (2018)

Systematic review of supervision for nurses

N= 553 (search) / N=27 (full text and met eligibility) / N=15 (numerical data)

- No independent outcome measures
- Outcome exclusively based on supee report ONLY

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Line Management (LMS) vs Professional (PS) vs Clinical or Practice Supervision (CS)

- LMS = focus on appraisal, performance and overall delivery of a service
- PS = focus on professional role (professional standards, ethics and codes of conduct and team relationship) with supervisor evaluation
- CS = focus on practitioner's individual practice which may include some self evaluative process

ALL exclude team dynamics issues

- Different processes NOT necessarily different individuals
- Separation of agendas and relationships
- Recognition of "feed" from one role to the other

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Legal Responsibilities Of Supervisors

BACP (2021)

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Legal Responsibilities Of Supervisors

- Negligence leading to damage when there is duty of care
- Clients can create "contributory negligence" by their own actions
- Organisations have a "vicarious liability" for their employees actions
- Supervisors are 3rd parties who share responsibility
- Responsibility: inexperience > experienced/qualified practitioners
- Responsibility of ALL practitioners to ascertain insurance AND contractual compatibility
- Courts will use current written professional and government standards to make judgements
- "NICE Guidelines are likely to constitute a responsible body of opinion for the purposes of litigation"
- Majority of decisions are settled out of court. Few examples of case law eg Phelps vs Hillingdon LBC (2000)

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Legal Responsibilities Of Supervisors

Key Learning Points

- Supervision in the counselling professions in England N.Ireland & Wales BACP Good Practice in Action 032 Legal Resource (2021)
- Seek advice on individual circumstances with your insurance provider AND ensure your supervision role is covered

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Five Supervision Models

- Proctor Functional Model (1986/1992)
- Stoltenberg & Delworth (Integrated Developmental Model-IDM) (1987/2006)
- Hawkins & Shohet (Double Matrix Model) (1989/2006)
- Waskett 4S Solution Focused Model (2009)
- Milne's CBT Model (2000/2017)

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Proctor's 3 Function Interactive Model (1986/1992)

- **FORMATIVE FUNCTION** (Learning) – clinical supervision concerned with developing the skills, abilities and understandings of the supervisee
- **RESTORATIVE FUNCTION** (Support) – clinical supervision concerned with how the supervisee responds emotionally to the stresses of the work
- **NORMATIVE** (Accountability) – clinical supervision concerned with maintaining and ensuring the safety and effectiveness of the supervisee's practice work

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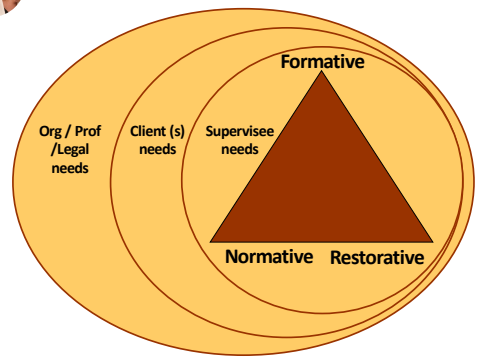

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Examples of differential stakeholder needs in supervision

- **Supervisee needs** : Developmental interests, immediate sources of anxiety regarding clients
- **Client needs:** Interventions that the supervisee is not that interested in, type or length of involvement that is impractical or challenging
- **Organisational needs:** Boundaries on length of involvement, other service pressures, governance requirements

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Initial supervision discussions

- Key question: What is the purpose of this supervision relationship ?
- Proctor's functions within the triple contexts and negotiated between supervisor and supervisee
- Additional factors include private vs organisational, educational vs professional maintenance/reaccreditation
- Identify key goals in each function PLUS timescale before review
- Link subsequent meeting agendas to key goals EITHER via supervisee raised issues OR by supervisor introducing them into the agenda

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Exercise

How would you approach the application of the adapted Proctor Model to your work with supervisees in setting up supervision and what issues might it raise?

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Stoltenberg & Delworth

(Integrated Developmental Model-IDM) 1987/1998/2006

- Focus on supervisee's development from "novice to expert" WITHIN training
- Supervisor's role is to facilitate supervisee's developmental movement using strategies appropriate to their current developmental level
- Identification of strategies appropriate to developmental stage
- Identification of areas to address
- Repeated research failure to validate developmental stages

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Integrated Developmental Model

Stoltenberg and Delworth

Level	Motivation	Autonomy	Self-other awareness
1	High motivation High anxiety Focus on skills acquisition	Dependent on supervisor, need for structure, direct feedback, minimal direct confrontation	Limited self-awareness. Focus on self: anxiety performance Difficulty seeing strengths / weakness
2	Fluctuating. More complexity shakes confidence.	Dependency-autonomy conflict. Specific help Dependent/ evasive	Focus more on client, can empathise. May become enmeshed, need balance
3	Stable Remaining doubts not disabling Total professional identity and how therapist role fits	Firm belief own autonomy Sense of when necessary to seek consultation	Accepts strengths/ weaknesses Can focus on client and process info. Including use of own reactions.

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Stoltenberg and Delworth's Domains of Professional Activity

- Intervention skills competence
- Assessment techniques
- Interpersonal assessment
- Client conceptualization
- Individual differences
- Theoretical orientation
- Treatment goals and plans
- Professional ethics

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Stoltenberg's Supervision Styles

Collaboration

- Facilitative
- Confrontative
- Conceptual
- Prescriptive
- Catalytic

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Supervision Styles

Facilitative: nurturing atmosphere; validation, warmth, liking, allow reflection and introspection, convey trust

Confrontive: highlights discrepancies; examine and compare (inc. emotions) achieve congruence beliefs, and behaviours

Conceptual: Discussion around theories, principles, Integrate theory and research; substantive content; give meaning, analytical thinking to events, tie together isolated events

Prescriptive: specific plan of action; give guidance; direct intervention; prescribe to ensure client welfare; treatment or specific instructions. Aims to satisfy dependency and eliminate certain behaviours

Catalytic: promote change; stir things up, promote reflection, encourages integration/ enhances meaning

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Styles & Developmental Levels

Level 1
General considerations: Provide structure and keep anxiety at manageable levels
Client assignment: Mild presenting problems or maintenance cases
Interventions: Facilitative, prescriptive, conceptual, catalytic (late Level 1 & 2)
Mechanisms: Observation (video or live), Skills training, Role playing, Interpret dynamics (limited; client or trainee), Readings, Group supervision, Appropriate balance of ambiguity/conflict, Address strengths, then weaknesses, Closely monitor clients

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Styles & Developmental Levels

Level 2
General considerations: Less structure provided; more autonomy encouraged, particularly during periods of regression or stress
Client assignment: More difficult clients and severe clients & confidence shaken
Interventions: Facilitative, Prescriptive: only occasionally, Confrontational, Conceptual, More alternative views, Catalytic: process comments, highlight countertransference & affective reactions to client or supervisor
Mechanisms: Observation (video or live), Role playing (although less important than at Level 1), Interpret dynamics (see "Catalytic," above), Parallel process, Group supervision, Broader clientele

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Styles & Developmental Levels

Level 3
General considerations: Most structure provided by trainee, more focus on personal and professional integration and career decisions
Client Assignment: N/A
Interventions: Facilitative, Confrontive: Occasionally necessary, Conceptual: From personal orientation, Catalytic: In response to blocks or stagnation
Mechanisms: Peer supervision, Group supervision, Strive for integration

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Styles & Developmental Levels

Level 1	Level 2	Level 3
Facilitative Prescriptive & Conceptual Catalytic (Confrontive)	Facilitative (Prescriptive) Conceptual Catalytic	Facilitative (Confrontive) Conceptual (Catalytic for blocks/ Stagnation)

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Supervision Styles

- **Facilitative**

"I can see you're feeling really worried about whether you're going in the right direction with this client"

" Tell me about the things you thought went well with the session"

"I know that you're unsure whether to do A or B. Both seem viable. Why not try one and see"

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Supervision Styles

- **Prescriptive**

Statement based: Information giving or direct description or advice on intervention

"Generally speaking GAD clients use worry as a safety behaviour, so this needs to be the next step in your treatment plan"

"Ask directly about the client's family context. Do you feel confident about how to do this or do you need some help?"

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Supervision Styles

- **Conceptual**

"What does the Ehlers & Clark model suggest we do at this point?"

"How would relational frame theory treat this clients belief differently from 2nd wave cognitive theory?"

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Supervision Styles

- **Confrontive**

Negative feedback **e.g.**

- "It's important that you assess risk with clients that present like this. You must do that next time"
- I feel you are spending too much time exclusively focusing on relationship building and that you need to start challenging the client over this issue"

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Supervision Styles

- **Catalytic**

Draw attention to process micro-issues inc.

"If you look at the way you're describing your client here, I'm wondering if you fully capture their cultural background and its effects"

" What thoughts went through your mind when your client failed to complete their therapy homework for the second time and how did that influence what you did next"

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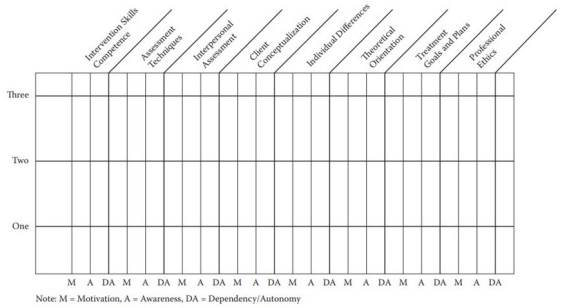
Stoltenberg Assessment & Mapping



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Exercise

Bring to mind one or more supervisees. **Write down** a concrete example of something **APPROPRIATE** that you could say to them in supervision from **EACH** of the styles and share with your partner your thoughts about communicating this to your supervisee

- Facilitative (Validating and trust building)
- Prescriptive (Information giving or direct description or advice on intervention)
- Conceptual (Theoretical or policy position question)
- Confrontative (negative feedback)
- Catalytic (draw attention to attitudes/thoughts on case)

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Case Discussion

- Criteria for HOW cases chosen determined by function of supervision
- For FORMATIVE function consider:
  - Themed cases
  - Success cases
  - Slow change cases
- Supervisor should give guidance template on how to present a case

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Exploratory case discussion methods

Lewin-Kolb

Solution Focused

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Exploratory case discussion methods

Advantages

- Allows cross-modality and cross-professional supervision
- Supervisee led development and support using supervisee strengths

Disadvantages

- Possibly problematic for very inexperienced supervisees
- Doesn't provide the advantages of formative OR normative didactic methods

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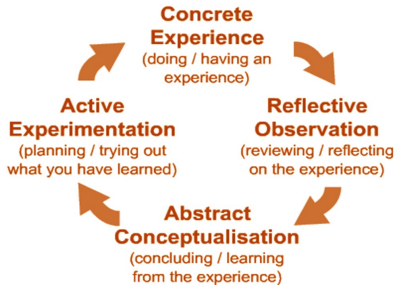
Lewin-Kolb Experiential Learning Circle & Supervision



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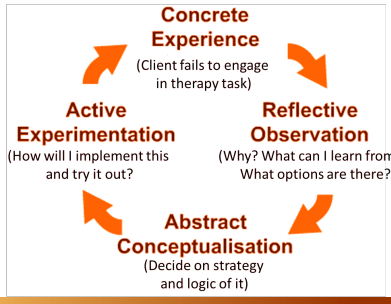
Example of Lewin-Kolb Experiential Learning Circle & Supervision



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Demonstration of use of the Lewin-Kolb Learning Circle

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Solution Focused Supervision Waskett (2006)

Assumes therapist's development and restoration comes from identifying and using their strengths
Focuses on these strengths that the therapist already has.
Pragmatic - helps the therapist notice what works — their skills, abilities, creative ideas, etc. in the service of the client/patient.
Assumes that the therapist is ALWAYS doing something right by preventing things being worse than they are
A question asking NOT a didactic style
Invites the therapist to articulate their preferred future in terms of being as good a therapist as they can possibly be for the client
Uses scales and circular questioning to note and measure progress towards the therapist's best practice "What did you/they do, to do that"

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Solution Focused Supervision Sample Questions

Beginning:

- What would you like to have happen in the next hour in supervision, so you'd know it was worth your while coming?
- How will you know afterwards that today's meeting was useful?

Approaching a case or issue:

- What strengths did you demonstrate in the issue or case we're about to discuss?
- Tell me about times when you're not experiencing difficulties with this client & what's different at those times

Listening to a case description:

- When you did X, how did you know that might be a useful direction to take?

When the practitioner focuses on the client:

- What alternatives went through your mind before you decided to do X?
- What would the client like to see you do differently and how might they respond differently?

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Solution Focused Supervision Micro-Skills Questions

At descriptions of specific interventions:

- What were you trying to accomplish there?
- How would that have been useful?
- With hindsight, what might you have done differently and how might the client have responded differently?

When noticing the client's improvement or change:

- How did the client manage to do that?
- What do you think the client found helpful (in you) and how did you manage to do that?

Near the end:

- On a scale of 0 — 10, where 10 = the best supervision session you could possibly have and 0 = the opposite, where would you rate this session? Why not 0 and how might we move up one in future?

Adapted from Briggs & Miller (2005)

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Demonstration of use of the Solution Focused Case Discussion

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Exercise

With your partner, share the role of supervisor and supervisee – 15 mins each way round.

Take a real life (but anonymised) case you've worked with as the material. The first supervisor should explore it using the Lewin Kolb, the second supervisor should explore their partner's case using a Solution Focused approach.

Prompts are contained on the exercise sheet

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Hawkins & Shohet (Double Matrix 7 Eyed Model)

- Describes the different systemic influences on supervisory activity
- Designed to stimulate multiple perspectives which increase learning and increase opportunities for change in the supervisee behaviour through the parallel process of supervision

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Hawkins & Shohet (Double Matrix 7 Eyed Model)

Matrix 1
Exploration of therapy session

Mode 1: Focus on supervisee's (SE) description of content session & client narrative / behaviour (to separate from interpretation/preconceptions)
Mode 2: Exploration & reflection on SE's interventions in the session (what done/not done and why)
Mode 3: Exploration and reflection on SE / client relationship and session process

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Hawkins & Shohet (Double Matrix 7 Eyed Model)

Matrix 2
Exploration of therapy session

Mode 4: Focus on SE's own reactions to client and its implications for working with the client

Mode 5: Parallel process – is the SE reacting to SR the way the client reacted to SE?

Mode 6: SR's counter transference to SE

Mode 7: Focus on organisational, cultural, social, ethical and contractual context of therapy & supervision

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Therapy Models Of Supervision

Why do they exist and what are their advantages and disadvantages?

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Therapy Models Of Supervision

A modelling opportunity for supervisees!

Advantages	Disadvantages
Rated most valuable aspect by supervises	….only because supervisees rarely get other opportunities
Modelling and practice THE most effective method of skill development	….yet role play rarely used by the majority of supervisors
	Supervision is not therapy structurally (purpose)
	Supervision is not therapy experientially (power differential)
	Modality specific so problematic for cross modality/professional supervision

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CBT Model Of Supervision

Milne (2006/2017)

- Examination of supervision from 1st principles
- Primary focus on formative supervision and trainees and reaccreditation (normative)
- Clarity of structure (overall purpose, case discussion focus, discussion structure and skill development and evaluation)

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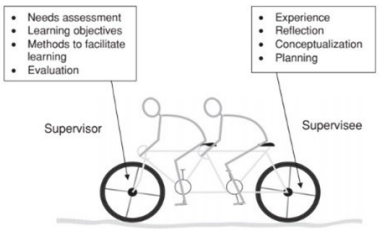
CBT Model of Supervision



Tandem Model of Supervision Milne & James (2005)

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CBT Model of Supervision

Step	
1	Clarify the supervision question. *Aim for a clear question which will promote learning.*
2	Elicit relevant background information. *Keep it brief and structured, e.g. client problem statement, key points of history, formulation and progress to date.*
3	Request an example of the problem. *This will usually include listening to a session tape extract.*
4	Check supervisee's current understanding. *This establishes their current competence and gives an indication of the 'learning zone' where supervision should operate.*
5	Decide the level or focus of the supervision work. *For example, a focus on micro-skills, or problem conceptualization, or on problematic thoughts and feelings within therapist.*
6	Use of active supervision methods. *Role-play, modelling, behavioural experiment, Socratic dialogue.*
7	Check if the supervision question has been answered. *Encourage the supervisee to reflect and consolidate the learning.*
8	Format a client-related action plan. *Formalize how the learning will be used within the therapy.*
9	Homework setting. *Discuss any associated development needs, e.g. reading related literature or self-practice of a CBT method.*
10	Elicit feedback on the supervision. *Check for any problems in the supervision alliance, or learning points for the supervisor.*

Gordon (2012)

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CBT Supervision

Additional Key Points Of Interest

- Therapist cognitions as obstacles rather than insights
- Focus on a range of formative methods inc. demonstration, role play and behavioural experiments

Table 3.1 The main methods of supervision within 24 reviewed studies (Milne *et al.*, 2008).

	Studies (N)	*Studies (prevalence, %)*
1. Training (teaching; skills instruction)	18	75
2. Feedback	15	63
3. Observing (live or recorded)	10	42
4. Goal-setting	9	38
5. Question and answer (information-gathering; clarifying)	9	38
6. Modelling (demonstration, live or video/audio)	7	29
7. Planning (managing agenda; arranging next meeting)	6	25
8. Reinforcement/praise/support	6	25
9. Discussion	5	21
10. Prompts (verbal and written reminders, handout, etc.)	5	21
11. Review/reflection	5	21
12. Role-play	5	21
13. Explanation (rationale provided; socialization to model)	4	17
14. Monitoring client benefit	4	17
15. Summarizing	4	17
16. Challenging	3	13
17. Self-disclosure	3	13
18. Listening	2	8
19. Problem-solving	2	8
20. Rehearsal of skills	2	8

Source: Milne, D.L. (2009).

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Exercise

Use Gordon's protocol for case discussion with a partner (supervisor and supervisee). The supervisor should act as time keeper and complete the discussion within 15 mins

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Practicalities

- Mandatory vs. optional?
- One to one vs. group vs. both?
- Frequency and length?
- Contracts, ground rules and protocols
- Uni-professional vs. cross-professional?
- Physical resource allocation – the Zoom challenge
  Waskett (2006)
- Agenda Setting & Structuring Sessions
- Records

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Contracts

- Legal standing (BACP 2021)
- Address mutual expectations and possible ambiguities
- Simple language > Complicated legal language
- Reviewed annually OR at start of new placements/jobs/roles

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Contracts: Royal College of OTs (2015)

- Who the agreement is between.
- Who holds copies of the agreement.
- When the agreement will be reviewed.
- The types of supervision that might be used: e.g. hands-on demonstration / close overseeing of a task, one-to-one meetings, group meetings etc.
- What model might be used.
- The frequency and length of supervision meetings.
- The location for supervision meetings.
- Who takes responsibility for organising the supervision meetings.
- In what limited circumstances supervision might be cancelled.
- The content of supervision, what may or may not be brought to a supervision meeting by either party, also how much
- What each party might need to prepare prior to any meeting.
- Who will record formal supervision, and how.
- How records will be kept.
- What will happen to the record if the supervisee leaves, changes job or has a different supervisor.
- The extent of confidentiality and when or how it might/should be broken.
- What the supervisee can expect or have a right to.
- What the supervisor can expect or have a right to.
- What the supervisee's responsibilities are.
- What the supervisor's responsibilities are.
- How conflict, or the inability to maintain the agreement, will be managed.
- Reference to a local supervision policy if present.

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Uniprofessional vs Cross Professional Unimodality vs Cross Modality

1. Recognise whether this issue applies to your situation
2. Identify legitimate material and aims of the relationship & boundaries (communicate to all parties)
3. Recognise how professional/identity differences influence dynamics
   - historical relationship (higher/lower/competitive status)
   - importance of deference and validation as outsider
   - ? discover alternative sources of shared identity
   - issue amplified in groups

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Supervision Agenda Setting

- Personal update,
- Session agenda setting
- Link to last session
- Previously supervised cases
- Check on homework tasks
- Discussion of agenda items,
- Assignment of new homework,
- Summary from clinical supervisor
- Feedback from supervisee

(Liese and Beck 1997)

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Supervision Agenda Setting

- In-line with supervision goals
- Collaborative (contributed to by both supervisee & supervisor)
- Time boundaried items?
- Organise items by:
  - (1) Easy
  - (2) Most important
  - (3) Success items

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Records

- Supervision records are legally an extension of client records
- Requirement for length of time keeping secure and releasing to appropriate authorities
- Importance shared agreed record for supervisor and supervisee
- Implications for confidentiality
- Anonymised records?

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Sample Action Sheet Principles

(1) Only record outcomes OR outcome plus bullet point process
(2) Identify person responsible for action
(3) Identify review / feedback date
(4) Record of any other professional body / organisational requirement e.g. CTS-R live supervision assessment
(5) Signed & dated by both parties

NO self reflective diary entries
NO requirement for minutes format

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Example of Recording in Supervision

Action/Decision/Advice	Person(s) Responsible	Completion / Feedback or Review Date
1. Raise motivation issue with client DF ▪ Erratic attendance ▪ Time limited therapy ▪ Current life changes prevent focus	Supvee	15 February
2. Supvee reassured re: concerns regarding client FF ▪ Lack of progress ▪ Concerns abt this expressed by client ▪ Difficult nature of problem reiterated by Supvor	Supvor	6 March

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Supervision records are confidential to the supervisor/supervisee however; they do remain the property of the Trust. Occasionally, under special circumstances, supervision notes are required as evidence that supervision has been delivered or as evidence to demonstrate personal development/learning to a governing body, regulatory/inspectorate body or other similar organisation. It should be noted that a court has the power to subpoena records in any and all circumstances if they have relevance to a matter before the judge.

Black Country NHS Trust (2018)

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Group vs. Individual Supervision

- Originates with Freud's "formative" supervision groups
- Peer supervision groups have largely arisen for logistical reasons only
- Requires a "group skill set" for the group / supervisor

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Group Supervisor Skills

- **Potential Uses of Group Supervision**
  - Demonstrations / Skill practice
  - Group discussion of cases
- **Potential Limitations of Group Supervision**
  - Individual attention to cases
  - Normative functions of supervision
  - Supportive supervision for individualised need

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Group Supervisor Skills

- Need to view the group as the supervisee NOT as a group of supervisees
- Need to recognise importance of typical group processes e.g. competition/support
- Need to use an exclusively facilitative style to encourage the group to use case examples as a learning process
- Need to resist individual facilitative or consultative style

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Peer Supervision

Benefits	Disadvantages
• Insight into novel approaches • Allows reflection and problem solving • Used throughout career • Allows peer review • Source of CPD • Offers reassurance • Prevents loneliness • Less effected by inappropriate power issues	• Can become TOO relaxed/ informal • Not enough challenge • Creation of myth of "power-free" group • Can focus & importance • Fails to address normative issues • Only addresses supportive issues common to the group

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BPS Approved Certificate in Clinical Supervision – Next Steps:

- Essential Supervision Skills (2 day foundation course) – Accreditation Level 1 – this course
- Interactive Online Exam – Accreditation Level 2
- Advanced Supervision Skills (1 day follow-up course) – Accreditation Level 3 – LIVE
- Final Assignment plus delegate's supervision record – Accreditation Level 4

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SCAN ME

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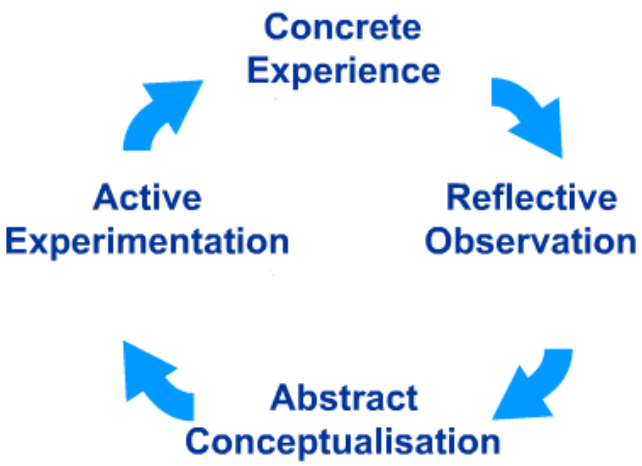
Lewin Kolb Exercise



Solution Focused Exercise

Beginning:

What would you like to have happen in the next hour in supervision, so you'd know it was worth your while coming?

How will you know afterwards that today's meeting was useful?

Approaching a case or issue:

What strengths did you demonstrate in the issue or case we're about to discuss?

Tell me about times when you're not experiencing difficulties with this client & what's different at those times

Listening to a case description:

When you did X, how did you know that might be a useful direction to take?

When the practitioner focuses on the client:

What alternatives went through your mind before you decided to do X?

What would the client like to see you do differently and how might they respond differently?

At descriptions of specific interventions:

What were you trying to accomplish there?

How would that have been useful?

With hindsight, what might you have done differently and how might the client have responded differently?

When noticing the client's improvement or change:

How did the client manage to do that?

What do you think the client found helpful (in you) and how did you manage to do that?

Near the end:

On a scale of 0 — 10, where 10 = the best supervision session you could possibly have and 0 = the opposite, where would you rate this session? Why not 0 and how might we move up one in future?

SUPERVISEE LEVELS QUESTIONNAIRE – REVISED

Answer the items that follow in terms of your own current behavior. In responding to the items, use the following scale:

Never	Rarely	Sometimes	Half the time	Often	Most of the time	Always
1	2	3	4	5	6	7

1. I feel genuinely relaxed and comfortable in my counseling sessions.

1	2	3	4	5	6	7

2. I am able to critique counseling tapes and gain insights with minimum help from my supervisor.

1	2	3	4	5	6	7

3. I am able to be spontaneous in counseling, yet my behavior is relevant.

1	2	3	4	5	6	7

4. I lack self-confidence in establishing counseling relationships with diverse client types.

1	2	3	4	5	6	7

5. I am able to apply a consistent personalized rationale of human behavior in working with my clients

1	2	3	4	5	6	7

6. I tend to get confused when things don't go according to plan and lack confidence in the ability to handle the unexpected.

1	2	3	4	5	6	7

7. The overall quality of my work fluctuates; on some days I do well, on other days I do poorly.

1	2	3	4	5	6	7

8. I depend on my supervision considerably in figuring out how to deal with my clients.

1	2	3	4	5	6	7

9. I feel comfortable confronting my clients.

1	2	3	4	5	6	7

10. Much of the time in counseling I find myself thinking about my next response instead of fitting my intervention into the overall picture.

1	2	3	4	5	6	7

11. My motivation fluctuates from day to day.

1	2	3	4	5	6	7

12. At times, I wish my supervisory could be in the counseling session to lend a hand.

1	2	3	4	5	6	7

13. During counseling sessions, I find it difficult to concentrate because of my concern about my own performance.

1	2	3	4	5	6	7

14. Although at times I really want advice/feedback from my supervisor, at other times I really want to do things my own way.

1	2	3	4	5	6	7

15. Sometimes the client's situation seems so hopeless, I just don't know what to do.

1	2	3	4	5	6	7

16. It is important that my supervisor allow me to make my own mistakes.

1	2	3	4	5	6	7

17. Given my current state of professional development, I believe I know when I need consultation from my supervisor and when I don't.

1	2	3	4	5	6	7

18. Sometimes I question how suited I am to be a counselor.

1	2	3	4	5	6	7

19. Regarding counseling, I view my supervisor as a teacher/mentor.

1	2	3	4	5	6	7

20. Sometimes I feel that counseling is so complex that I will never be able to learn it all.

1	2	3	4	5	6	7

21. I believe I know my strengths and weaknesses as a counselor sufficiently well to understand my professional potential and limitations.

1	2	3	4	5	6	7

22. Regarding my counseling, I view my supervisor as a peer/colleague.

1	2	3	4	5	6	7

23. I think I know myself well and am able to integrate that into my therapeutic style.

1	2	3	4	5	6	7

24. I find I am able to understand my clients' view of the world, yet help them to objectively evaluate alternatives.

1	2	3	4	5	6	7

25. At my current level of professional development, my confidence in my abilities is such that my desire to do counseling doesn't change much from day to day.

1	2	3	4	5	6	7

26. I find I am able to empathize with my clients feeling states, but sill help them focus on problem resolution.

1	2	3	4	5	6	7

27. I am able to adequately assess my interpersonal impact on clients and use that knowledge therapeutically.

1	2	3	4	5	6	7

28. I am adequately able to assess the client's interpersonal impact on me and use that therapeutically.

1	2	3	4	5	6	7

29. I believe I exhibit a consistent professional objectivity and ability to work within my role as a counselor without undue overinvolvement with my clients.

1	2	3	4	5	6	7

30. I believe I exhibit a consistent professional objectivity and ability to work with my roles a s counselor without excessive distance from my clients.

1	2	3	4	5	6	7

Scoring Key: Self and Other Awareness Items: 1, 3, 5, 9, 10*, 13*, 24, 26, 27, 28, 29, 30
Motivation Items: 7, 11*, 15*, 18*, 20*, 21, 23, 25
Dependency-Autonomy Items: 2, 4*, 6*, 8, 12*, 14, 16, 17, 19*, 22
*Indicates reverse scoring. To score, sum the items in the scale and divide by the number of items

Developed by C. Stoltenberg. Unpublished version of Supervisee Levels Questionnaire.

Background Reading

Hawkins & Shoet (2012) Supervision in the Helping Professions

Stoltenberg and McNeill (2010) IDM Supervision

Waskett, C. (2006) The SF Journey. Therapy Today; 17: 2, 40–42

Milne & Reiser (2017) A Manual for Evidence-Based CBT Supervision

Proctor, Brigid (1986) 'Supervision: A co-operative exercise in accountability' in A. Marken & M Payne (eds) Enabling and Ensuring: Supervision in Practice Leicester National Youth Bureau/Council for Education and Training in Youth and Community Work